PROBLEM

What was happening — and why it mattered

WHAT WAS HAPPENING

Physio is WorkSafeBC's largest healthcare spend ($80M of $500M total). Extension requests — authorizing continued treatment beyond initial approval — arrived via fax, phone, and mail. 139,380 historical requests processed, with 79% always approved and 27% resolved same-day, raising concerns that case managers weren't reviewing details.

WHO WAS AFFECTED

Injured workers waiting for treatment authorization continuity; physiotherapists and office admins re-submitting via fax; case managers skimming requests they should be adjudicating; Healthcare Programs managing $80M physio spend.

WHY IT MATTERED

Extensions directly enable patient care — without timely approval, treatment stops. But rubber-stamp approvals on $80M in spend suggested the workflow wasn't supporting clinical review. The product had to digitize requests while strengthening — not bypassing — case manager judgment.

DISCOVERY

How I approached discovery

USER INTERVIEWS

Physiotherapist focus groups and provider feedback

Physios don't want to fill out functional abilities on web forms — they want to describe treatment progress in text. Body map (Form 6) adds no value for providers who already know injury anatomy.

WORKFLOW OBSERVATIONS

PXT app usage analysis

PXT app is used by office admins, not physiotherapists — but surveys were filled out by physios, creating bias in satisfaction scores (contract terms vs. tool quality).

STAKEHOLDER INTERVIEWS

Healthcare Programs (Gabi), case managers, CMS team

Physio extensions are 16% of annual spending concern. Case managers need to be more involved in accept/decline — auto-approve would defeat the business purpose.

ANALYTICS

139,380 historical request analysis

79% always approved (109,693/139,380) sparked auto-approve debate. Aging data showed most requests resolve within 3 days — ping feature deprioritized. 27% same-day resolution raised review quality concerns.

RESEARCH

Multi-channel request mapping (portal, fax, mail)

Physio extensions don't only come from the portal — fax/mail channels require holistic CMS architecture, not portal-only design.

PRODUCT JUDGMENT

“We segmented providers by workflow (office admin submits, physio treats) instead of organization type — behavior predicted adoption and survey bias better than demographics.”

CURRENT WORKFLOW

Before — and where it broke

Legacy extension workflow — treatment continuity blocked by manual channels



PRODUCT JUDGMENT

“We rejected auto-approve despite 79% approval rate because high approval can indicate broken review (skimming), not unnecessary review — on $80M spend, judgment must be enabled, not bypassed.”

PRODUCT THINKING

Observation → insight → decision

DECISION CHAIN 1

OBSERVATION	INSIGHT	DECISION	TRADEOFF	EXPECTED OUTCOME
Senior PMs and architects pushed to auto-approve extensions since 79% are always approved.	Auto-approve defeats the business purpose — case managers need to be more involved because physios want to keep treating and WSBC pays significant insurance on extensions.	Reject auto-approve; design for case manager review with better information capture.	Slower processing vs. stronger clinical oversight on $80M spend category.	Case managers receive structured digital requests with treatment rationale — enabling informed review, not rubber-stamping.

DECISION CHAIN 2

OBSERVATION	INSIGHT	DECISION	TRADEOFF	EXPECTED OUTCOME
Stakeholders wanted body map (Form 6) for provider extension requests.	Providers know injury anatomy — body map adds scope without value. Text box suffices for phase 1 treatment progress description.	Deprioritize body map; simple text field for extension rationale.	Less visual data for case managers vs. faster launch and lower provider friction.	Reduced scope and provider completion time; avoided translation complexity for CMS display.

DECISION CHAIN 3

OBSERVATION	INSIGHT	DECISION	TRADEOFF	EXPECTED OUTCOME
Existing physio extension flow lacked end-to-end provider experience — no decision rationale surfaced.	Providers made phone calls because they couldn't see case manager decisions or reasoning — increasing support burden.	Prioritize PRQ enhancements: request tracking, decision rationale from CMS, notification emails, error validation.	Delayed new request types to fix existing physio extension experience first.	Funding secured for CMS optimization narrative: fix current workflows before adding new request types.

DECISION CHAIN 4

OBSERVATION	INSIGHT	DECISION	TRADEOFF	EXPECTED OUTCOME
Injured worker information was re-entered for each request type in the portal.	Providers submit multiple request types per patient — redundant data entry increases errors and abandonment.	Redesign flow: injured worker info entered once, then select request type.	Cross-product workflow change requiring coordination with other POs and security team.	Developers connect one injured worker form to service type page instead of six separate connections.

DECISION CHAIN 5

OBSERVATION	INSIGHT	DECISION	TRADEOFF	EXPECTED OUTCOME
Payee number manually entered caused production incident (wrong payee# submission).	Pre-populating payee# requires authentication — scope cut deferred this but incident validated the need.	Document in backlog; scope cut for phase 1 with manual entry and error validation.	Data entry error risk vs. authentication dependency and BCeID migration timeline.	Error validation features shipped; payee pre-population noted for future authenticated release.

PRODUCT JUDGMENT

"We deprioritized the body map for providers because they already know injury anatomy — text box sufficed for phase 1 without adding CMS translation scope."

KEY PRODUCT DECISIONS

Decision matrix

KEY PRODUCT DECISIONS

Decision	Alternative	Why we chose this	Tradeoffs	Outcome
Reject auto-approve for extensions HUMAN VS AI	Auto-approve up to a spending threshold	79% approval rate doesn't mean review is unnecessary — case manager involvement is the business purpose on $80M physio spend.	Slower processing vs. clinical oversight and spend control	Product de to strengthe review qua bypass it
Prioritize decision rationale over new request types PROCESS VS SOFTWARE	Add new request types before fixing physio extension experience	Existing flow incomplete without CMS decision rationale — providers phone in because they can't see decisions.	Delayed feature expansion vs. end-to-end functional experience	PRQ fundi secured w reprioritiza narrative
Deploy PRQ enhancements early (before CMS decision rationale) SPEED VS SAFETY	Wait until April for full CMS + portal launch together	View Details page and error validation are value-add now; early feedback and incident buffer before decision rationale launch.	Phased launch complexity vs. earlier provider value and risk mitigation	Select feat deployed I decision ra targeted fo launch
Text box over body map for extension rationale USER EXPERIENCE VS COMPLIANCE	Implement Form 6 body map for providers	Providers know injury anatomy; body map adds CMS translation scope without provider value.	Less visual injury data vs. faster launch and lower friction	Phase 1 sh with text-b treatment descriptio
Deprioritize ping feature; prioritize callback request CONFIGURATION VS PRODUCT	Build both ping (stuck request alert) and callback features	Aging data showed most requests resolve within 3 days — ping adds little value; callback streamlines physio communication.	Fewer provider communication tools in v1	Developm capacity re to higher-i callback w

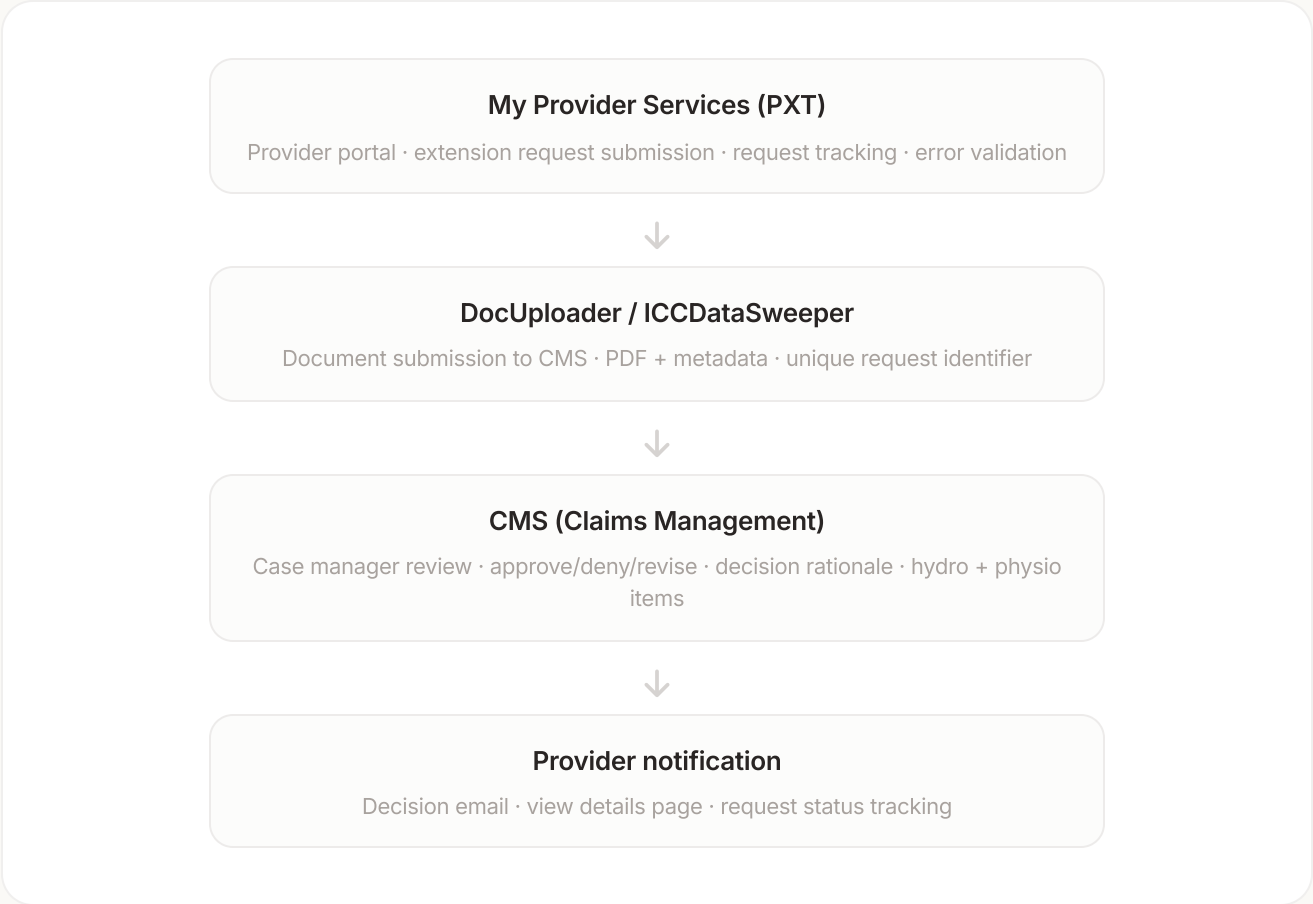
PRODUCT JUDGMENT

“We prioritized decision rationale over new request types because providers were calling in due to lack of end-to-end experience — fix the care continuity loop before expanding.”

TECHNICAL COLLABORATION

Architecture at product level

Product-level architecture — multi-channel provider requests



PRODUCT RESPONSIBILITIES

- PRQ feature prioritization and funding narrative
- Cross-PO coordination (PXT + CMS teams)
- Provider workflow redesign (injured worker info once)
- Data-driven feature cuts (ping vs. callback, body map vs. text)

ENGINEERING RESPONSIBILITIES

- PXT portal extension request forms
- DocUploader integration for CMS submission
- Request ID linking between online and CMS sides
- Error validation and status display logic
- CMS decision rationale and notification email pipeline

SHARED RESPONSIBILITIES

- Multi-channel architecture (portal + fax + mail) with CMS team
- Status testing matrix (all combinations of physio/hydro decisions)
- Pod transfer knowledge transfer (Pioneers → Carebears)
- Security review and authentication roadmap

Phased launch strategy (early deploy vs. full CMS launch)

TECHNICAL TRADEOFFS

DocUploader pattern for physio vs. new flexible pattern for future request types (Michael/Chitanya architectural decision)

Online passes unique identifier to CMS for linking — fax/mail channels need holistic design

Authentication deferred for payee# pre-population — manual entry with error validation for phase 1

CMS surfaces decision data only (not full submission) for future request types

PRODUCT JUDGMENT

“We designed for multi-channel reality (portal + fax + mail) instead of portal-only — physio extensions don't only come from the portal.”

EXECUTION

How we shipped

TIMELINE

2023–2024: PXT physio extension request digital submission

2024: $300K approved for provider expansion; CMS team $60–80K

Mar 5: PXT deployment with select PRQ features

TODO: full PRQ launch with CMS decision rationale (targeted early April)

Security review delayed final launch by ~1 week

PRIORITIZATION

Fix existing physio extension experience before new request types

Decision rationale + request tracking over PDF auto-extraction

Callback request over ping feature (data-driven)

Text box over body map for phase 1

Injured worker info-once workflow redesign

MVP (PRQ PHASED)

Digital 60-day physio extension request submission

Request tracking on provider portal

Error validation for submission fields

View Details page for providers

Decision rationale + notification email (CMS-dependent phase)

RISKS

CMS team discovery scope creep (1.5 months vs. capped budget)

Pod transfer friction (Pioneers → Carebears)

Multi-channel architecture complexity (portal + fax + mail)

Payee# data entry errors without authentication

Security vulnerabilities impacting PRQ story points

TRADEOFFS ACCEPTED

Phased launch over big-bang CMS + portal release

Manual payee# entry over authentication dependency

Text rationale over body map visualization

CMS optimization before new request type expansion

CROSS-FUNCTIONAL PARTNERS

CMS team (Mimi) — decision rationale, case manager workflow

Architects (Michael, Chitanya, Rajat) — multi-channel linking

Pioneers → Carebears dev pods (Tony, Madhu, Zak)

Healthcare Programs (Gabi) — spend and clinical oversight

Security team — authentication and vulnerability review

PRODUCT JUDGMENT

“We deployed PRQ enhancements early for provider feedback and incident buffer rather than waiting for full CMS launch — phased value delivery over big-bang release.”

RESULTS

Outcomes

BUSINESS OUTCOMES

$80M

PHYSIO SPEND ADDRESSED

Largest healthcare spend category; extensions are major cost driver

OPERATIONAL OUTCOMES

139K+

HISTORICAL REQUESTS ANALYZED

133,390 completed; data informed auto-approve debate and feature prioritization

USER OUTCOMES

Mar 5

PXT DEPLOYMENT

Select PRQ features deployed; digital extension requests live on provider portal

BUSINESS OUTCOMES

PRQ

FUNDING SECURED

Reprioritization narrative: optimize current workflows before new request types

OPERATIONAL OUTCOMES

TODO

DIGITAL VS FAX ADOPTION

Post-launch channel shift metrics not yet documented

USER OUTCOMES

TODO

CM REVIEW QUALITY IMPROVEMENT

Structured digital requests intended to enable informed review vs. skimming

PRQ Feature 3: Track requests — provider visibility into submission status

PRQ Feature 4: Decision rationale surfaced to providers via CMS

PRQ Feature 5: Error validation to reduce submission defects

PRODUCT JUDGMENT

“We used aging data to deprioritize the ping feature — most requests resolve within 3 days, so alerting on stuck requests added little value vs. callback communication.”

LESSONS LEARNED

What changed how I think

What surprised me

79% approval rate was used to argue for auto-approve — but high approval rate can mean the review workflow is broken (skimming), not that review is unnecessary.

What I would do differently

Cap CMS team discovery scope earlier with explicit budget constraints — open-ended discovery on a $300K budget created timeline risk for Carebears dev capacity.

How this changed how I think as a PM

Care delivery products require distinguishing between 'automate the approval' and 'enable better judgment.' On $80M in treatment spend, the product's job is to make case manager review more informed — not to remove it.

WHY THIS MATTERS

Relevance to healthcare product leadership

Treatment extensions sit at the intersection of care delivery and operational workflow — they directly determine whether injured workers continue receiving physiotherapy. This project required balancing clinician usability, case manager clinical oversight, multi-channel architecture, and spend control — the same product leadership needed for healthcare workflow products where patient outcomes depend on operational systems working correctly.

QUESTIONS THIS PROJECT FORCED ME TO ANSWER

Head of Product questions — and how I answered them

Is this actually a software problem?

Partially. The approval behavior is a process problem (skimming), but digitizing requests and surfacing decision rationale is a software problem that enables better clinical review.

Should this be solved with process, configuration, software, or AI?

Software for request submission, tracking, and decision transparency. Process change (CM review quality) follows from better information capture — not from removing review.

Who actually owns this workflow?

Split: providers submit (PXT), case managers adjudicate (CMS), architecture spans both. Required cross-PO alignment between PXT and CMS teams.

What evidence changed our direction?

139K request analysis (79% approve, 27% same-day), aging data (deprioritize ping), provider feedback (text over body map), production incident (payee# errors).

Where should humans remain in control?

Case managers must approve/deny/revise every extension — including separate decisions for physio and hydro items. Auto-approve was explicitly rejected.

What would happen if we built nothing?

Providers continue faxing; case managers continue skimming; injured workers face treatment gaps; $80M physio spend lacks structured review support.

IF I DID THIS PROJECT AGAIN

Knowing what I know today

What would I change?

Involve CMS team in portal design from day one with joint architecture sessions — multi-channel reality should shape portal UX, not be discovered during integration.

What would I validate earlier?

Survey respondent role (admin vs. physio) — discovering bias late meant prior satisfaction data was unreliable for product decisions.

What assumptions were incorrect?

Assumed high approval rate meant automation was safe. Healthcare Programs clarified that approval rate indicated review quality failure, not automation opportunity.

What would I intentionally NOT build?

Auto-approve for extensions — and PDF auto-extraction for phase 1 (introducing new service digitally mattered more than UX optimization of data entry).

How would AI change my approach today?

AI could assist case managers with treatment progress summarization from structured requests — but would not replace approve/deny judgment. Prescriptive models considered but required forms with clinical context.

How would I measure success differently?

Track CM review time per request and decision reversal rate — not just digital adoption. Success is better judgment, not faster rubber-stamping.

What would I prototype before implementation?

Case manager review interface with decision rationale — validate that structured digital requests actually change review behavior before building provider-side features.

HEAD OF PRODUCT VIEW

If you only read one thing — why trust this PM with a healthcare workflow?

PROBLEM

$80M physio spend with extensions submitted via fax; case managers skimming approvals on treatment decisions that directly affect injured worker care.

JUDGMENT

Rejected auto-approve despite 79% approval rate. Prioritized decision transparency and CM review quality over automation or feature expansion.

HARDEST TRADEOFF

Phased PRQ launch (provider features before CMS decision rationale) vs. big-bang release — chose early provider value and incident buffer.

TECHNICAL CHALLENGE

Multi-channel architecture (portal + fax + mail) requiring unique request ID linking between PXT and CMS — not a portal-only problem.

HEALTHCARE COMPLEXITY

Treatment continuity for injured workers; separate physio/hydro decisions; clinical oversight on largest spend category; provider-admin workflow split.

BUSINESS OUTCOME

PRQ funding secured; PXT deployed March 5; digital extension requests live. TODO: adoption and review quality metrics.

PRODUCT LESSON

In care delivery workflows, optimize for better judgment — not faster approval. High approval rates can signal broken review, not unnecessary review.